



# Before the Fall: Connecting High-Risk Adults to Prevention Before Emergency Department Presentation

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## WHY IT MATTERS

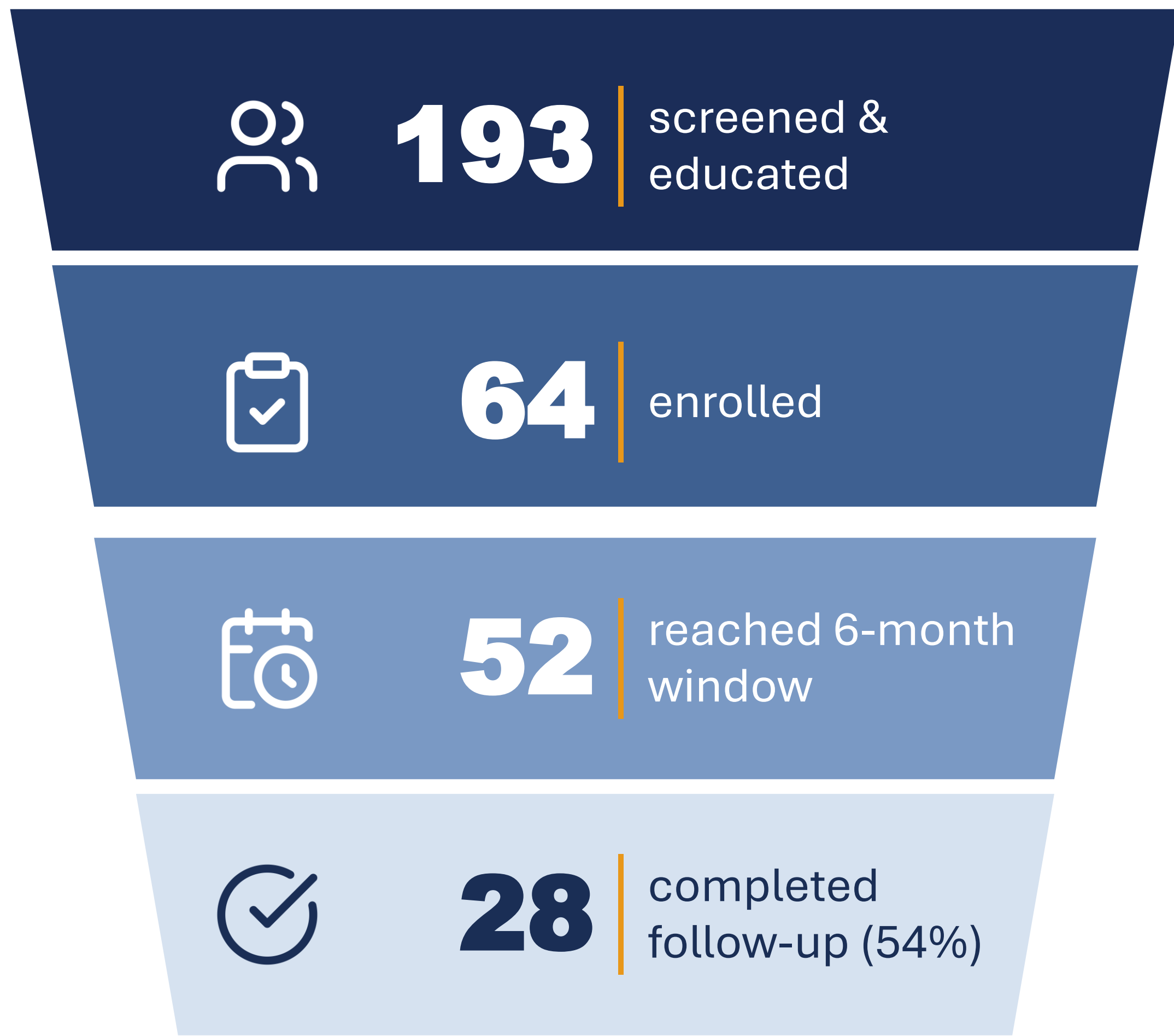
- Unintentional falls drive **~8.9 million U.S. emergency department visits** each year.<sup>1</sup>
- The CDC's **Stopping Elderly Accidents, Deaths, and Injuries (STEADI)** is an evidence-based screen–assess–intervene framework.<sup>3</sup>
- Multifactorial and multiple-component interventions **can reduce fall rates**.<sup>2</sup>
- Evidence-based exercise **programs reduce falls**.<sup>5</sup>
- Validated prevention exists, but **delivery in primary care remains inconsistent**.<sup>4</sup>

## PILOT OBJECTIVES

- Reach** high-risk community-dwelling adults.
- Characterize** fall risk, concern, and prior prevention engagement.
- Intervene** with motivational interviewing, tailored education, and community linkage.
- Assess** 6-month prevention engagement, acute-care utilization, and falls.
- Evaluate** feasibility to inform a hybrid type 1 trial.

## PARTICIPANTS

5 events | 2023–2026 | Rural & metro  
Mean age 73.6 ± 9.8 y | 75.0% female



## FROM SCREENING TO PREVENTION

**15% → 46%**

**reported fall-prevention class participation** at 6 months  
*among paired respondents (n=26)*

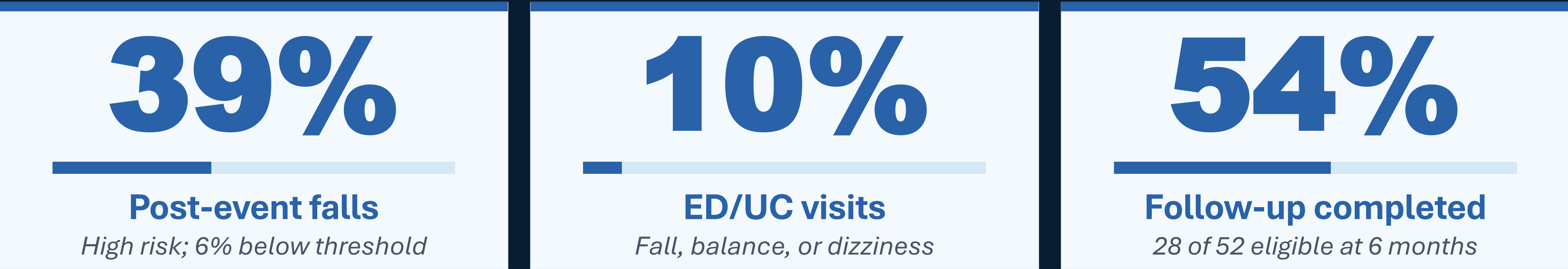
**Two-thirds screened high risk, yet most had little prior prevention engagement**

## PILOT RESULTS AT A GLANCE

### AT ENROLLMENT

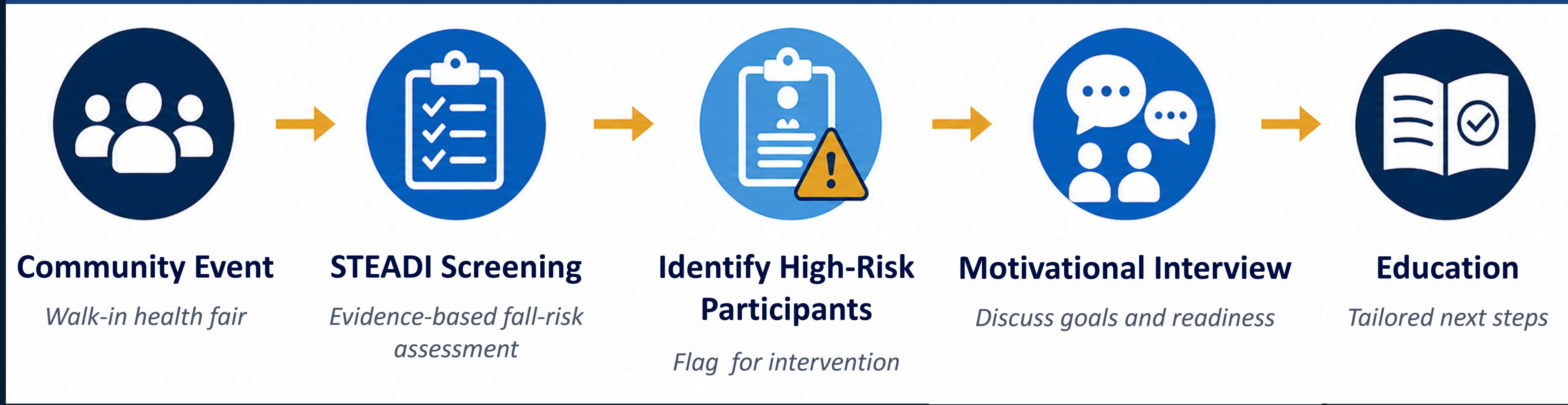


### AT 6-MONTH FOLLOW-UP



## IMPLEMENTATION MODEL

### SCREENING-TO-INTERVENTION PATHWAY



## WHAT WE FOUND

- 66.7%** screened high risk  
*STEADI ≥4; 42/63*
- 54.7%** fell in the prior year  
*76.2% of high-risk participants*
- 57.8%** no prior PCP discussion
- 87.5%** never attended a class
- 85.9%** moderate-to-high concern
- 15.4% → 46.2%** fall-prevention class participation  
*Baseline to 6 months; paired, n=26*
- 39.4% vs 5.6%** post-event falls  
*High risk vs below; RR 7.09 (95% CI 1.01–49.9)*
- 9.6%** post-event fall-related ED/UC visits

## WHAT THIS MEANS

- The model reached adults with high risk, high concern, and little prior prevention engagement and **delivered individualized fall-prevention intervention**
- Prevention engagement increased** post-event
- Pilot **feasibility, targeting, and outcome-rate estimates** will inform the design of a larger trial

## LIMITATIONS

- Self-selected pilot**; no comparison group.
- Limited follow-up** among eligible participants
- Fall ascertainment relied on **self-report and EHR review**; out-of-system events may be **missed**
- STEADI validated for **ages ≥65; 11 were younger**
- English-only consent **limited enrollment**

## FUTURE DIRECTIONS

- Continue and expand** community fall-prevention events and community-partner linkage
- Geospatially target** rural, high-burden Oregon communities.
- Hybrid Type 1 trial** testing prevention uptake, falls, and fall-related acute-care utilization.

## References:

- CDC National Center for Injury Prevention and Control. Web-based Injury Statistics Query and Reporting System (WISQARS). Accessed Oct 1, 2026.
- Hopewell S, et al. Community fall-prevention interventions. *Cochrane Database Syst Rev*. 2018;7(7):CD012221.

- Stevens JA, Phelan EA. Development of STEADI. *Health Promot Pract*. 2013;14(5):706-714.
- Burns ER, et al. PCP fall-prevention discussion. *Prev Med Rep*. 2018;9:149-152.
- Karlsson L, et al. Community-based fall-risk screening. *J Geriatr Phys Ther*. 2020;43(4):185-193.

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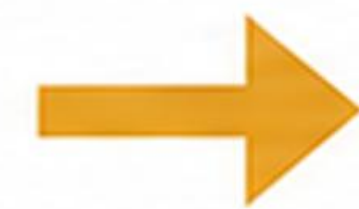
### Community Event

*Walk-in health fair*



### STEADI Screening

*Evidence-based fall-risk  
assessment*



### Identify High-Risk Participants

*Flag for intervention*



### Motivational Interview

*Discuss goals and readiness*



### Education

*Tailored next steps*



# PARTICIPANTS

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Mean age  $73.6 \pm 9.8$  y | 75.0% female



**193**

screened &  
educated



**64**

enrolled



**52**

reached 6-month  
window



**28**

completed  
follow-up (54%)



FROM SCREENING TO PREVENTION

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*among paired respondents (n=26)*

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**67%**

Screened high risk

**58%**

No PCP discussion

**86%**

Fall concern

**39%**

Post-event falls

**10%**

ED/UC visits

**54%**

Follow-up completed



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